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Title: Lumetra (pelvaglutide) injection: Prescribing Information
Product: Lumetra

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LUMETRA (pelvaglutide) injection, for subcutaneous use

BOXED WARNING

WARNING: RISK OF THYROID C-CELL TUMORS

- In rodents, pelvaglutide causes dose-dependent thyroid C-cell tumors. It is unknown whether Lumetra causes thyroid C-cell tumors, including medullary thyroid carcinoma (MTC), in humans (5.1).
- Lumetra is contraindicated in patients with a personal or family history of MTC or in patients with Multiple Endocrine Neoplasia syndrome type 2 (MEN 2) (4).

1 INDICATIONS AND USAGE

Lumetra is a glucagon-like peptide-1 (GLP-1) receptor agonist indicated as an adjunct to diet and exercise to improve glycemic control in adults with type 2 diabetes mellitus.

Limitations of use:

- Not for use in patients with type 1 diabetes mellitus or for the treatment of diabetic ketoacidosis.
- Not studied in patients with a history of pancreatitis.
- Not indicated for chronic weight management.

2 DOSAGE AND ADMINISTRATION

2.1 Recommended dosage

Step	Dose	Duration
Initiation	0.5 mg once weekly	4 weeks (not effective for glycemic control)
Maintenance	1 mg once weekly	At least 4 weeks before any increase
Additional glycemic control	2 mg once weekly	Maximum recommended dose

Administer once weekly on the same day each week, at any time of day, with or without meals.

2.2 Missed dose

If a dose is missed, administer it as soon as possible within 4 days (96 hours) after the missed dose. If more than 4 days have passed, skip the missed dose and give the next dose on the regularly scheduled day.

2.3 Changing the weekly injection day

The day of weekly administration can be changed if the time between two doses is at least 3 days (72 hours).

2.4 Administration

- Inject subcutaneously in the abdomen, thigh or upper arm. Rotate injection sites with each dose.
- Use a new needle for each injection. Never share a Lumetra pen between patients, even if the needle is changed.
- Inspect visually before use: the solution should be clear and colorless. Do not use if particles or discoloration are seen.
- When using Lumetra with insulin, give separate injections; never mix them.

3 DOSAGE FORMS AND STRENGTHS

Injection: clear, colorless solution in single-patient-use prefilled pens. Each pen delivers 4 doses.

- Pen delivering 0.5 mg per injection (blue label)
- Pen delivering 1 mg per injection (green label)
- Pen delivering 2 mg per injection (purple label)

4 CONTRAINDICATIONS

- Personal or family history of medullary thyroid carcinoma or Multiple Endocrine Neoplasia syndrome type 2.
- Serious hypersensitivity reaction to pelvaglutide or any of the excipients.

5 WARNINGS AND PRECAUTIONS

5.1 Risk of thyroid C-cell tumors

Counsel patients about the potential risk of MTC and the symptoms of thyroid tumors (a mass in the neck, dysphagia, dyspnea, persistent hoarseness).

5.2 Acute pancreatitis

Acute pancreatitis, including fatal and non-fatal hemorrhagic or necrotizing pancreatitis, has been observed with GLP-1 receptor agonists. If pancreatitis is suspected, discontinue Lumetra and do not restart if confirmed.

5.3 Hypoglycemia with insulin secretagogues or insulin

Patients receiving Lumetra with an insulin secretagogue (such as a sulfonylurea) or insulin have an increased risk of hypoglycemia. Consider lowering the dose of the sulfonylurea or insulin when starting Lumetra.

5.4 Acute kidney injury

Acute kidney injury has been reported, usually in patients with nausea, vomiting or diarrhea leading to dehydration. Monitor renal function in patients reporting severe gastrointestinal adverse reactions.

5.5 Diabetic retinopathy complications

Rapid improvement in glucose control has been associated with a temporary worsening of diabetic retinopathy. Monitor patients with a history of diabetic retinopathy.

5.6 Acute gallbladder disease

Cholelithiasis was reported in 1.4% of Lumetra-treated patients and 0.6% of placebo-treated patients. If cholecystitis is suspected, gallbladder studies are indicated.

6 ADVERSE REACTIONS

6.1 Clinical trials experience

Table 2. Adverse reactions in at least 5% of Lumetra-treated patients (pooled placebo-controlled trials, 40 weeks)

Adverse reaction	Placebo (N=592) %	Lumetra 1 mg (N=601) %	Lumetra 2 mg (N=598) %
Nausea	6.1	14.7	18.2
Diarrhea	5.4	8.6	10.1
Vomiting	2.3	6.9	9.4
Decreased appetite	1.8	4.2	6.0
Constipation	3.0	5.1	5.8
Abdominal pain	3.4	5.0	5.6

Gastrointestinal adverse reactions were most frequent during dose escalation and decreased over time. Injection site reactions occurred in 2.0% of Lumetra-treated patients. Discontinuation because of gastrointestinal adverse reactions occurred in 3.9% of patients on 2 mg and 0.8% on placebo.

7 DRUG INTERACTIONS

7.1 Oral medications

Lumetra delays gastric emptying and may affect the absorption of oral medications taken at the same time. Monitor drugs with a narrow therapeutic index, such as warfarin, when starting Lumetra.

7.2 Insulin secretagogues and insulin

See 5.3.

8 USE IN SPECIFIC POPULATIONS

8.1 Pregnancy

Limited data in pregnant women are insufficient to determine a drug-associated risk. Discontinue Lumetra at least 2 months before a planned pregnancy because of the long washout period of pelvaglutide.

8.2 Lactation

There are no data on the presence of pelvaglutide in human milk. Consider the benefits of breastfeeding together with the mother's clinical need for Lumetra.

8.4 Pediatric use

Safety and effectiveness have not been established in patients younger than 18 years.

8.6 Renal impairment

No dose adjustment is recommended for patients with renal impairment. Monitor renal function in patients with severe gastrointestinal adverse reactions (5.4).

8.7 Hepatic impairment

No dose adjustment is recommended for patients with hepatic impairment.

12 CLINICAL PHARMACOLOGY

12.3 Pharmacokinetics

Parameter	Value
Absolute bioavailability (subcutaneous)	About 86%
Time to peak concentration	1 to 3 days
Elimination half-life	About 7 days
Steady state	Reached after 4 to 5 weekly doses

14 CLINICAL STUDIES

In the 40-week PELVA-2 trial, HbA1c fell from a baseline of 8.2% by 1.4 percentage points with Lumetra 1 mg and 1.8 percentage points with Lumetra 2 mg, compared with 0.3 percentage points with placebo.

16 HOW SUPPLIED, STORAGE AND HANDLING

Pen	Doses per pen	NDC
0.5 mg per injection	4	99107-310-01
1 mg per injection	4	99107-311-01
2 mg per injection	4	99107-312-01

Each carton contains 1 pen and 4 single-use needles.

Storage:

- Before first use: store in a refrigerator at 2°C to 8°C (36°F to 46°F). Do not freeze. Do not use a pen that has been frozen.
- After first use: the pen can be stored for up to 56 days at room temperature up to 30°C (86°F) or in a refrigerator at 2°C to 8°C.

- Keep the pen cap on when not in use to protect from light. Remove the needle after each injection and store the pen without a needle attached.
- Discard the pen 56 days after first use, even if it still contains medicine.

17 PATIENT COUNSELING INFORMATION

- Inform patients of the potential risk of thyroid C-cell tumors and the symptoms to report.
- Advise patients to stop Lumetra and contact their healthcare provider if they have severe abdominal pain that does not go away, with or without vomiting.
- Advise patients to drink fluids to avoid dehydration if they have nausea, vomiting or diarrhea.

REVISION HISTORY

Version	Effective date	Changes
2.1	2025-11-15	5.6 acute gallbladder disease added; 2.3 changing the injection day clarified.
2.0	2025-03-01	2 mg dose added (2.1, 6.1, 14).

Medical Information: 1-800-555-0199, medinfo@larkmoor.example. To report suspected adverse reactions, contact Larkmoor Therapeutics Pharmacovigilance at 1-800-555-0177.

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